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Evaluating the Prevalence of Suicide Risk Screening Practices in Accredited Hospitals

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Background: The Joint Commission's National Patient Safety Goal (NPSG) on suicide prevention (NPSG.15.01.01) requires accredited hospitals to screen all patients aged 12 years and older who are being evaluated or treated for behavioral health conditions as their primary reason for care for suicidal ideation using a validated screening tool. Some hospitals have expanded screening to include nonbehavioral health care patients.

Methods: This cross-sectional observational study explored the prevalence and challenges of suicide risk screening practices among Joint Commission–accredited hospitals. An online questionnaire was sent to 859 general medical/surgical hospitals. Chi-square tests were used to evaluate differences in response rates, and responses were adjusted by hospital characteristics (bed capacity, location, system, and teaching status).

Results: A total of 284 (33.1%) hospitals responded. The majority ($n = 225$ [79.2%]) reported screening all patients hospitalwide, and 185 (65.1%) had implemented a suicide prevention framework that includes protocols for positive screens and risk assessment. Challenges for implementing a comprehensive universal suicide risk screening and assessment protocol included insufficient staffing and lack of secure environments for at-risk patients. Of the 59 organizations not conducting hospitalwide screening, 94.9% indicated multiple reasons, including negative impact on workflow (30 [50.8%]), burden on providers (30 [50.8%]), not a requirement (29 [49.2%]), and workflow feasibility (28 [47.5%]).

Conclusion: Results suggest the majority of accredited hospitals have implemented suicide risk screening practices that exceed current Joint Commission requirements. The lack of sufficient resources to adequately address patients who screen positive for suicide risk remains a key challenge to universal screening.

Increased recognition of suicide as a preventable public health issue has pointed to the need for a broader approach to suicide prevention beyond behavioral health care settings.[1] Two studies found that nearly 30% of individuals who died by suicide had a health care visit during the seven days before suicide, and approximately 50% made a medical visit four weeks before suicide.[2,3]

In July 2019 The Joint Commission, a not-for-profit health care accrediting organization, revised its National Patient Safety Goal (NPSG) on suicide prevention (NPSG.15.01.01) to improve suicide risk screening and assessment practices. This NPSG requires accredited hospitals (including hospital outpatient departments accredited under the hospital) to screen all patients aged 12 years or older who are being evaluated or treated for behavioral health conditions as their primary reason for care for suicidal risk using a validated screening tool.[4] While the NPSG requires screening for behavioral health care patients, some hospitals have expanded suicide risk screening beyond behavioral health care patients, noting increasing acceptability and feasibility of universal suicide risk screening.[5–7] Implementation of screening can be accomplished with existing validated screening tools, which vary in terms of their target population, feasibility, and quality of underlying evidence.[8]

Suicide risk screening using an evidence-based tool is a vital first step in implementing a suicide prevention program. Effective suicide risk screening is implemented within a framework that includes protocols for positive screens, risk assessment, and intervention based on patient needs. Universal screening alone does not decrease suicidal behavior unless it is combined with a protocol that includes risk assessment and an intervention, such as post-visit telephone contact.[9] For this reason, The Joint Commission NPSG requires that, following a positive screen, organizations use an evidence-based process to assess suicidal ideation, plan, or intent; suicidal or self-harm behaviors; and other risk and protective factors. Ideally, a suicide risk assessment is completed by a licensed mental health provider. It includes a comprehensive evaluation, addresses the patient's suicide risk factors, provides initial diagnosis, and informs development of treatment and follow-up plans in collaboration with the patient and parent/guardian.[10]

Organizations are expected to document patients' overall level of risk for suicide and implement plans to miti-

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gate the risk. The organization is also expected to follow and monitor the implementation and effectiveness of policies and procedures to address the care of patients identified as at risk for suicide, including guidelines for training and competency of staff who care for these patients, reassessment, patient monitoring, and follow-up care at discharge. Reluctance to adopt universal screening has largely centered on the quality of evidence for screening tools, lack of robust sensitivity/specificity, and concerns related to resources required to follow up on positive screens.[11–13]

The purpose of this study was to determine the prevalence of expanded suicide risk screening practices—exceeding current Joint Commission requirements—and to understand challenges and barriers associated with universal risk screening implementation.

METHODS

This cross-sectional observational study was part of a larger study that examined the prevalence of universal screening, as well as leading postdischarge suicide prevention and quality improvement activities.[14] The work was guided by a scientific advisor and six expert advisors who helped develop the questionnaire (Appendix 1, available in online article) and advised on data analysis and interpretation.

To classify a hospital as applying comprehensive universal screening and risk assessment protocol, the following conditions were required:

- Screening all patients hospitalwide, including patients whose primary reason for care is not a behavioral health condition
- Implemented a clinical protocol for suicide risk screening, a protocol for positive screens, and a protocol for conducting risk assessment.

This project was reviewed by Ethical and Independent Review Services and determined exempt from Institutional Review Board (IRB) review.

Surveyed Hospitals

To explore the prevalence of suicide risk screening practices exceeding current Joint Commission requirements, the sample focused on general medical/surgical hospitals (Note: Because accredited psychiatric hospitals are required to screen all individuals served, these facilities were excluded from this study.) To be eligible for inclusion in the study, hospitals must have received accreditation during 2020–2021 ($n = 1{,}333$). Hospitals due for accreditation in 2022–2023 were excluded to reinforce that the study was unrelated to accreditation. Specialty hospitals, such as long-term acute care and rehabilitation hospitals, were excluded due to limited representation in the hospital population ($n = 185$). A total of 859 hospitals were eligible for inclusion.

Data Collection

In August 2022 an advance invitation letter was sent to all eligible hospitals. Two weeks later, an e-mail with a project information sheet and a link to a 40-item electronic questionnaire (on the Qualtrics XM platform; Qualtrics LLC, Provo, Utah) was sent to hospital quality department directors, who were asked to complete the questionnaire or forward it to the person(s) with institutional knowledge related to the organization's suicide prevention efforts. Four follow-up reminders were sent to encourage questionnaire completion. Data collection closed on November 1, 2022. The desired minimum sample size, calculated based on 5% precision and confidence intervals (CIs) of 95% after applying a finite population correction factor, was determined to be 266 hospitals.

Data Analysis

Quantitative analyses were completed using R, version 3.5, software.[15] Sampling weights were used to adjust results for nonresponse. To determine weights, logistic regression was used to estimate the probability that a sampled hospital had completed the survey as a function of bed size category (< 100 beds, 100–299 beds, ≥ 300 beds), location (urban, rural), system status (system member, freestanding), and teaching status (major, minor, nonteaching). The inverse of the predicted probability of response from the logistic regression was used as the weight. Mean scores for screening practices, both overall and stratified by the hospital characteristics, were calculated, and weighted scores are provided. A p value less than $\alpha = 0.05$ was considered significant. Chi-square tests were used to test for differences in response rates by each hospital characteristic. Fisher's exact test was used if any of the expected counts were less than five. The questionnaire included four open-ended questions. Two researchers read each open-ended response and independently identified themes based on the question asked. After discussion and consensus, all identified themes were included as findings for each of the free-text questions.

RESULTS

Overall, 284 of 859 eligible hospitals (33.1% response rate) completed the questionnaire, exceeding the sample size needed for estimated precision. Characteristics of responding hospitals were similar to hospitals in the sample (Table 1). Universal screening rates are displayed in Table 2. Of 284 responding hospitals, 225 (79.2%) reported screening all patients hospitalwide (including the emergency department [ED]), and 185 (65.1%) hospitals met the full criteria for conducting comprehensive universal suicide-risk screening and assessment. Urban hospitals were significantly more likely than rural hospitals ($p < 0.001$) to have implemented comprehensive universal screening.

Table 1. Characteristics of Responding Hospitals Compared to Nonresponding Hospitals

Hospital Characteristic	Responders *n* (% of 284)	Nonresponders *n* (% of 573)	*p* Value
Bed Size			
< 100	86 (30.3)	203 (35.4)	0.32
100–399	151 (53.2)	283 (49.4)	
≥ 400	47 (16.5)	87 (15.2)	
System Membership			
Yes	222 (78.2)	489 (85.3)	0.009
No	62 (21.8)	84 (14.7)	
Primary Service			
CAH	31 (10.9)	68 (11.9)	0.011
General	214 (75.4)	463 (80.8)	
Other	39 (13.7)	42 (7.3)	
Location			
Rural	63 (22.2)	143 (25.0)	0.37
Urban	221 (77.8)	430 (75.0)	
Teaching Status			
Major	29 (10.2)	44 (7.7)	0.45
Minor	144 (50.7)	303 (52.9)	
Nonteaching	111 (39.1)	226 (39.4)	
Ownership			
For profit	53 (18.7)	106 (18.5)	0.99
Not for profit	187 (65.8)	376 (65.6)	
Government	44 (15.5)	91 (15.9)	

CAH, critical access hospital.

Table 2. Characteristics of Hospitals That Implemented Universal Suicide Risk Screening

Characteristic	Implemented Universal Screening; *N* = 185	Implemented Universal Screening; *N* = 185; Rate Weighted (%)	*p* Value
Location*			
Rural	31	49.0	< 0.001
Urban	154	69.1	
Teaching Status†			
Major	19	64.8	0.70
Minor	95	65.7	
Nonteaching	71	62.9	
System Member‡			
No	45	70.7	0.80
Yes	140	63.2	
Ownership			
For Profit	38	71.9	0.06
Government	30	60.6	
Not For Profit	140	63.1	

* Rural or urban location based on metropolitan statistical area (MSA) designation. A rural location is defined as located outside an MSA as designated by the US Office of Management and Budget. An urban area is a geographically defined, integrated social and economic unit with a large population nucleus.[1]

† Those with Council of Teaching Hospitals (COTH) designation. Minor teaching hospitals were those approved to participate in residency and/or internship training by the Accreditation Council for Graduate Medical Education (ACGME) or American Osteopathic Association (AOA), or those with medical school affiliation reported to the American Medical Association; nonteaching hospitals were those without COTH, ACGME, AOA, or Medical School (AMA) affiliation.[1]

‡ Indicates whether a hospital is affiliated with a health care system. A multihospital health care system is two or more hospitals owned, leased, sponsored, or contract managed by a central organization.[1]

Reference

1. American Hospital Association. AHA Hospital Statistics. A Comprehensive Reference for Analysis and Comparison of Hospital Trends, 2018 ed. Chicago: Health Forum, 2018.

Table 3. Suicide Risk Screening Practices

Suicide Screening Practices	*n* (% of 284)
Screen all patients hospitalwide.	225 (79.2)
Screen all patients in specific departments (for example, emergency, inpatient departments).	20 (7.0)
Screen some patient populations whose primary reason for care is not a behavioral health condition.	11 (3.9)
Only screen patients whose primary reason for care is a behavioral health condition.	28 (9.9)
Elements of Suicide Prevention Framework	*n* (% of 284)*
Clinical protocol for suicide risk screening	267 (94.0)
Protocol for positive screens	263 (92.6)
Protocol for risk assessment	244 (85.9)

* Respondents had the choice to make multiple selections (that is, select all that apply).

Fifty-nine hospitals (20.8%) indicated they were not conducting hospitalwide screening. Of these, 20 (7.0%) screened all patients in some departments, such as the ED; 11 (3.9%) screened some patient populations whose primary reason for care was not a behavioral health condition. A vast majority of hospitals (267 [94.0%]) use a clinical protocol for suicide risk screening. Most hospitals also have protocols for positive screens (263 [92.6%]) and risk assessments (244 [85.9%]) (Table 3).

Respondents not implementing universal screening were asked to indicate the likelihood of their hospital implementing hospitalwide suicide risk screening. Most of these 59 hospitals reported they were either extremely likely (19 [32.2%]) or somewhat likely (22 [37.3%]) to implement universal screening in the near future. Eighteen (30.5%) hospitals reported they were not at all likely to adopt universal screening.

When asked about factors that influenced hospitals' decisions not to implement universal suicide risk screening, 94.9% cited multiple reasons. These included some combination of anticipating negative impact on workflow (30 [50.8%]), increased burden on providers (30 [50.8%]), universal screening not being an accreditation requirement (29 [49.2%]), and feasibility of incorporating suicide risk screening into their workflow (28 [47.5%]) (Table 4).

Respondents that reported conducting universal screening were asked to indicate the degree to which their hospital found the process of suicide risk screening for patients to be challenging, ranging from "not at all challenging" to "extremely challenging." While 65 hospitals (28.9%) indicated that universal screening was "not at all challenging," the majority of hospitals (145 [64.4%]) implementing hospitalwide suicide risk screening found the process somewhat challenging, and an additional 13 (5.8%) found it extremely challenging. Approximately half of hospitals implementing universal suicide risk screening reported insufficient staffing to provide safety interventions (115 [51.1%]), unavailability of secure beds for people identified as being at risk of suicide (110 [48.9%]), and staff turnover (106 [47.1%]) as challenges their hospitals have encountered throughout the process (Table 5).

An open-ended question asked respondents to describe the processes by which their hospitals had overcome the challenges of implementing universal suicide risk screening. The most common responses included (1) targeting staff training, assessment, and recruitment and (2) standardizing the screening process (for example, integrating the screening tool into the electronic health record [EHR]). Other responses indicated the importance of leadership (for example, prioritization/commitment/support of a system initiative), identifying psychiatric safe spaces for at-risk patients, and collaboration across departments (for example, dedicated multidisciplinary team, telehealth utilization). When asked to describe reasons for the decision to screen all patients for suicide risk beyond the current Joint Commission requirements, they mentioned the high importance placed on patient safety, specifically the need to avoid the risk of missing a patient who might be in need of further assessment.

DISCUSSION

This study explored the prevalence of suicide risk screening practices among Joint Commission–accredited nonpsychiatric hospitals. Results suggest that the majority of accredited hospitals have implemented policies that exceed current requirements. Approximately two thirds of accredited general medical/surgical hospitals screen all patients hospitalwide and have clinical protocols for positive screens and risk assessment. This is an encouraging sign, as universal screening has been linked to enhanced suicide risk detection.[5,10,16–19] However, hospitals also reported challenges, including insufficient staffing and limited availability of ligature-resistant environments (including secure and dedicated patient beds as well as patient areas for people identified as being at risk for suicide). These issues were identified as concerns among respondents that have not adopted universal screening, and they are similar to issues identified in a previous study concluding that universal screening would create more overflow and affect ED capacity to care for behavioral health patients in secure, dedicated areas.[20]

It is worth noting that among some of the hospitals that reported screening all patients, universal screening did not

Table 4. Reasons for Not Implementing Universal Suicide Risk Screening

Reasons	*n* (% of 59)*
Anticipate negative impact on workflow (for example, slowed down clinical care)	30 (50.8)
Anticipate increased burden on providers	30 (50.8)
Universal suicide risk screening is not a standard requirement.	29 (49.2)
Feasibility of incorporating suicide risk screening into workflow	28 (47.5)
Anticipate negative impact on patient satisfaction due to repeated questions related to suicidality	22 (37.3)
Potential for incorrect information	20 (33.9)
Anticipate cost of extra providers (for example, cost of 1:1 observation)	15 (25.4)
Anticipate patients would be unmotivated to complete suicide risk screening	13 (22.0)
Other reasons (for example, patient privacy, reduced focus on at-risk patients, implementation difficulty, depression screening exists)	11 (18.6)
Provider legal liability concerns	7 (11.9)
Concerns about impact on minority and indigenous populations	2 (3.4)
Insurance authorization requirements	1 (1.7)
Missing	3 (5.1)

* Respondents had the choice to make multiple selections (that is, select all that apply); 95% of responding organizations indicated multiple reasons, including negative impact on workflow, burden on providers, not a requirement, and workflow feasibility.

Table 5. Challenges Associated with the Implementation of Universal Suicide Risk Screening.

To what degree has the hospital found the process of implementing universal suicide risk screening for patients challenging?	*n* (% of 225)
Not at all challenging	65 (28.9)
Somewhat challenging	145 (64.4)
Extremely challenging	13 (5.8)
Missing	2 (0.9)
Challenges of implementing universal suicide risk screening	***n* (% of 225)***
Insufficient staffing to provide safety interventions	115 (51.1)
Unavailability of secure beds for people identified as being at risk of suicide	110 (48.9)
Staff turnover	106 (47.1)
Patients provide incorrect information.	78 (34.7)
Increased burden on providers	77 (34.2)
Concerns about providing treatment or aftercare options for suicidality if identified	76 (33.8)
Unavailability of skilled providers to conduct timely risk assessment	69 (30.7)
Negative impact on workflow (for example, slowed down clinical care)	61 (27.1)
Patients unmotivated to complete suicide risk screening	51 (22.7)
Increased patient waiting time	45 (20.0)
Challenges related to electronic health records (EHRs) records and communication systems	45 (20.0)
Leadership changes or changes in leadership priorities	38 (16.9)
Negative impact on patient satisfaction	30 (13.3)
Provider legal liability concerns	27 (12.0)
Challenges of choosing an appropriate suicide risk screening and/or risk assessment tool	26 (11.6)
Other challenges (for example, lack of adequate staff and community resources, limited psychiatric beds)	20 (8.9)
Negative impact of cultural beliefs regarding suicide among patients	17 (7.6)
Insurance authorization requirements	12 (5.3)
We do not have a suicide clinical care pathway built in the EHR.	12 (5.3)
Availability of patient records for staff conducting risk assessment	11 (4.9)
Missing	13 (5.8)

* Respondents had the choice to make multiple selections (that is, select all that apply).

always include protocols for patients who screened positive for suicide risk, including risk assessment or follow-up. This is a critical gap, as screening provides little benefit if resources to follow up with adequate interventions are unavailable. As noted by Snyder et al., the process of determining, documenting, and providing care that is achieved through workflows and implementing protocols for patients who initially screen positive has been found to not only enhance the effectiveness of suicide screening, but to consequently promote efficient use of limited resources by reducing overresponse. Both factors are important for sustainability of universal suicide screening.[7]

Implementation of universal screening inevitably leads to detection of a greater number of individuals with potential suicide risk.[6,16,21,22] This is particularly important for individuals who have fewer interactions with the health care system, including rural and minority populations or individuals who may have unknown mental health con-

cerns.[10,23] Some experts have emphasized that organizational burdens of universal screening can be overcome and point to underutilization of best practices.[24] Various studies describe the use of tiered clinical pathways for patients who initially screen positive for suicide risk.[7,25] For example, a brief suicide safety assessment performed by trained providers can be used to classify acuity of suicide risk for patients who screen positive. Results of the brief assessment can help to determine next steps in patient care and application of further protocols depending on risk levels. Key questions, at this point, may include the following[25]:

- Would the patient benefit from additional mental health support and evaluation?
- Is it appropriate to discharge the patient home with detailed safety and follow-up planning?
- Should a more comprehensive suicide risk assessment be administered?
- Does the patient require immediate attention to keep them safe in the hospital?
- What are the hospital's protocols for safety precaution?

An evaluation of the cost-effectiveness of universal screening and intervention concluded that universal screening plus intervention as an economically optimal intervention depends on the value placed on outcomes by decision-makers.[26] Others have pointed to a study by Roaten et al.[5] as evidence that concerns about "excessively high positive screen rates that the hospital system would not be able to manage"[27(p. 1)] have been overstated.[5,27]

It is important to also examine our findings from the perspective of other studies that have pointed to perceived limitations of universal screening. For example, suicide screening instruments may have low predictive and sensitivity values in nonbehavioral health care settings, resulting in high numbers of false positives[13,28] and may put further strain on resources needed for risk assessment and potential intervention, a key concern for participants of this study. However, others have pointed out that the clinical value of existing screening tools is less about their low predictive and sensitivity values and more about the effectiveness of targeted suicide prevention interventions that determine outcomes for patients identified with increased risk.[13,29]

We could infer from the high percentage of hospitals that have already adopted universal screening practices that a key issue for them is the need to understand and address practical and economic challenges and barriers to successful implementation. Yet, having screening and assessment protocols in place does not mean clinical staff are actually performing the screenings with fidelity.[16] Recent systematic reviews have identified a significant gap in studies examining administrative or financial barriers to implementing universal screening or providing direction to effective implementation.[30,31] In the absence of such research or changes to payment models and incentives that can address these economic challenges, it is unclear if new mandates for universal screening in nonbehavioral health care settings would provide significant clinical benefit.

Our findings also indicate that organizations adopting universal screening place high value on avoiding the risk of missing patients in need of suicide screening, and many use EHR to help facilitate screening all patients. Identification and treatment of patients at risk for suicide remains an important safety issue. Given the pragmatic challenges to implementation of universal screening and subsequent risk assessment and safety planning for patients who screen positive, questions regarding practical implementation of more effective approaches remain. For example, research has shown that most patients who actually attempt suicide report being depressed, and reductions in suicidal behaviors occur after improved screening and treatment of depression in primary care, highlighting the value of improving screening depression in that setting.[1,13,28] A systematic review concluded there is need to train primary care physicians in depression recognition and treatment as evidence-based practices to prevent suicide.[1] This form of screening is distinct from universal suicide risk screening, but more research is needed on the barriers and facilitators to effective implementation in medical/surgical settings. Most recently, the US Preventive Services Task Force, while recommending screening for depression in the adult population, concluded there is currently insufficient evidence to support universal suicide risk screening in primary care.[32] A more recent study showed effectiveness of integrating suicide care in primary care using a combination of screening for depression, suicide screening, risk assessment, and safety planning in reducing risk of self-harm.[33] Specifically, rates of safety planning were significantly higher, and rates of documented suicide attempts significantly lower. These findings further underscore the significance of approaching screening within the context of quality safety planning and optimal care processes.

Limitations

This study has several important limitations. First, the study involved only Joint Commission–accredited hospitals. The implementation of The Joint Commission's NPSG on suicide prevention in 2019 has likely created a driver for accredited hospitals to adopt some suicide prevention practices that may not be widely implemented in nonaccredited organizations. Results, therefore, should not be generalized to nonaccredited hospitals. Second, nonresponders may differ in their suicide screening and prevention practices, and given that the survey was administered by The Joint Commission, nonresponders may have been reluctant to report practices viewed as unfavorable, while self-report and social desirability biases may have affected responding hospitals' answers to various questions. Our findings, therefore, may overestimate adoption of universal suicide risk screening practices. Finally, comprehensive universal suicide risk screening practices can be complex. Although respondents

were instructed to consult with the most knowledgeable person(s) in their organization, it is possible some respondents were unfamiliar with the extent of risk screening. In some cases, respondents may have reported implementing a practice for which there is a policy or procedure, even if that policy or procedure is not routinely followed. Results, therefore, may overestimate actual practices.

CONCLUSION

Overall, the findings indicate that although most accredited general medical/surgical hospitals recognize the value of universal screening and report screening practices that significantly exceeded current Joint Commission requirements, they have practical challenges related to implementation. The success of efforts to promote wider adoption of universal screening, risk assessment, and effective interventions for patients who screen positive for suicide risk will likely hinge on the ability to address these challenges. Additional research that examines effective implementation of other evidence-based practices such as depression recognition and treatment as part of suicide prevention efforts in primary care is also needed.

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SUPPLEMENTARY MATERIALS

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